

Pembrolizumab + carboplatin/paclitaxel → AC

Breast · NCCP 00857 · version 3

Your medicines

Pembrolizumab · Carboplatin · Paclitaxel · Doxorubicin · Cyclophosphamide

Your treatment schedule

Every 21 days · See page 2 for the treatment phases.

A picture of side effects

Tell your team about new or worsening symptoms. The same symptom can have more than one cause. Numbers connect each symptom box to its location.

Pembrolizumab · immune effects

Chemotherapy · phase-specific effects

1 Hormones / brain

Pembrolizumab

Unusual fatigue, dizziness, headache or thirst should be reported.

2 Lungs

Pembrolizumab

Report new cough or breathlessness promptly.

3 Heart

Doxorubicin / pembrolizumab

New chest symptoms, palpitations or breathlessness need assessment.

4 Liver / kidneys

Pembrolizumab

Blood tests monitor organ function. Report yellow eyes, dark urine or reduced urine.

5 Nerves

Paclitaxel

Report tingling or numbness before it persists or affects everyday activity.

6 Mouth / bowel

Chemotherapy / pembrolizumab

Report mouth soreness, persistent diarrhoea, vomiting or severe tummy pain.

7 Skin / nails

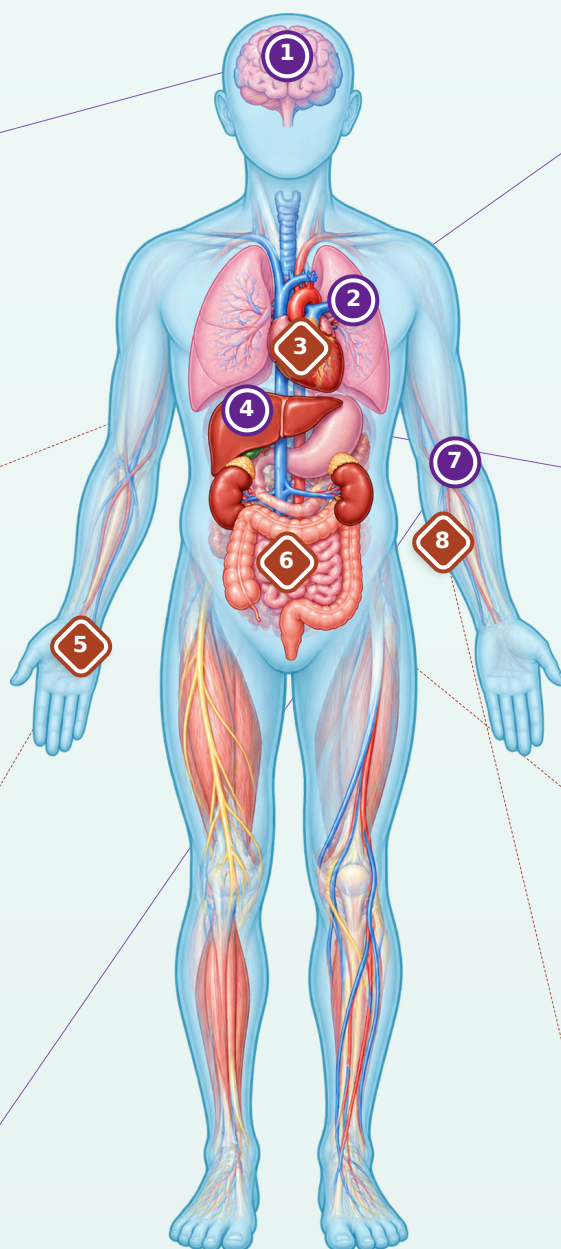
Chemotherapy / pembrolizumab

Rash, nail changes or an infusion reaction can occur. Severe or blistering rash needs urgent review.

8 Blood / infection

Chemotherapy

Fever or sudden illness needs immediate oncology contact. Medicines change between treatment phases.



This map shows selected effects to discuss; it does not show their likelihood or every possible side effect. Blood-count and whole-body effects are marked schematically.

Selected symptom patterns for discussion. Tell your oncology team about any new or worsening symptom. See page 2 for medicine-specific effects, monitoring and the portal QR.

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TREATMENT JOURNEY

Cycles 1-4 Every 21 days: pembrolizumab + carboplatin on day 1; paclitaxel on days 1, 8 and 15.	Cycles 5-8 Every 21 days: pembrolizumab + doxorubicin + cyclophosphamide on day 1.	Surgery Definitive breast surgery follows the neoadjuvant phase when clinically appropriate.	Adjuvant phase Pembrolizumab continues after surgery: 200 mg every 21 days for 9 cycles or 400 mg every 42 days for 5 cycles, according to the treatment plan.	Throughout Bloods and toxicity review guide whether treatment is given, delayed or modified.
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IMPORTANT MEDICINE-SPECIFIC EFFECTS

Pembrolizumab - immune effects - Immune inflammation can affect lungs, bowel, liver, hormone glands, kidneys, skin, heart, nerves or muscles. - New symptoms can begin during treatment or after treatment has stopped.	Paclitaxel - Peripheral neuropathy can cause numbness, tingling or pain. - Infusion reactions can occur despite premedication. - Low blood counts and hair loss are common treatment effects.
Carboplatin - Low blood counts, especially neutropenia and low platelets, can occur. - Allergic infusion reactions can occur, particularly after repeated exposure. - Nausea and vomiting may occur despite preventative medicines.	Doxorubicin + cyclophosphamide - Low blood counts and infection risk can be significant. - Doxorubicin can reduce heart function; cyclophosphamide can irritate the bladder. - Mouth soreness and hair loss are common treatment effects.

Other / shared treatment effects: The anatomy page highlights important symptom patterns for this exact regimen, but it cannot show every possible effect. Fatigue, nausea, appetite change, mouth/bowel symptoms or blood-count changes may occur depending on the medicines used. Report new or worsening symptoms even if they are not listed here.

Monitoring / medicines - Blood count, kidney/liver profile and glucose are checked regularly; thyroid testing is repeated during pembrolizumab treatment. - Paclitaxel requires premedication according to the regimen/local protocol. - Cardiac assessment may be required before the doxorubicin phase.	Bring / track - Your current medicines, including non-prescription products. - A note of new symptoms, when they started and what they stopped you doing. - Any temperature, blood-pressure or bowel-count readings your team asked you to record. - Questions about the purpose of treatment, scan/review timing or changes to the plan.
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WHEN TO CALL - DO NOT WAIT FOR THE NEXT APPOINTMENT

Call oncology urgently for fever, new breathlessness, persistent diarrhoea, jaundice, severe rash, significant bleeding, severe weakness or a new neurological symptom. For severe breathing difficulty, chest pain, collapse or seizure call 112/999. If your team has given you different local thresholds or emergency instructions, follow those instructions.

Consent discussion support

Use the approved consent process to document treatment aim, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

Regimen information only; this QR contains no patient details.

<https://sactcheck.com/patient/00857/>

Official NCCP 00857 v3 · SACTCheck v0.81.2 · patient information prototype for clinical review.

